

GENERAL CHARACTERISTICS OF THE SENTINEL GENERAL PRACTITIONERS (SGP) NETWORK IN BELGIUM: REPORT ON 2025

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WHO WE ARE

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As our name suggests, science and health are central to our mission. Sciensano's strength and uniqueness lie within the holistic and multidisciplinary approach to health. More particularly we focus on the close and indissoluble interconnection between human and animal health and their environment (the "One health" concept). By combining different research perspectives within this framework, Sciensano contributes in a unique way to everybody's health.

For this, Sciensano builds on the more than 100 years of scientific expertise.

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THE SENTINEL GENERAL PRACTITIONERS (SGP) NETWORK IN BELGIUM

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1. Background

Sentinel surveillance by general practitioners (GPs) is an accepted system of public health surveillance, i.e. the on-going collection, analysis, and interpretation of health data essential to public health practice, closely integrated with timely dissemination of information for intervention (1). In sentinel surveillance a sample of professionals or organizations such as general practices, hospitals, laboratories, clinicians are designated to collect and report data that are considered as representative for a specified population.

Networks of SGP were developed by GPs to use the wealth of the information they gathered in their daily practice for research, education or management. General practice is the first source of professional health information next to the general population and most health problems are exclusively managed by GPs (2;3). Almost all people in Belgium have a general practitioner and therefore they are a good source of information about the health of the population. In 1979 the Belgian network of SGP was developed drawing on experiences by the Weekly Returns Service in the UK and the Sentinel Stations in the Netherlands (4). In the early 1990s several networks of SGP were operating and co-operating in Europe (5;6).

All networks of SGP have in common that anonymous data from clinical practice on well-defined problems are recorded by voluntary, self-selected GPs covering a region/population and subsequently transferred to a central office for analysis and diffusion of study results. Networks of SGP may differ in their aim (surveillance of infectious/non-infectious health problems or oriented towards health services research), size (regional or national) and available methods for the estimation of populations at risk (patient lists or number of patient contacts per week) and data transmission (by paper forms or electronic data transmission) (5;6).

The **aims** of the Belgian SGP network have broadened over time. The network has a long history of routine surveillance by monitoring continuously the occurrence of influenza-like illness. The periodical surveillance of e.g. sexual transmitted infections (STIs) may be considered as an enhanced surveillance by its collection of additional data on top of age, gender and other core data. The surveillance of end-of-life care is definitely oriented towards health services research as the study aims to describe usual care by GPs (7;8). By providing epidemiological information as well as information on care provided in general practice, the SGPs support the policymakers in defining priorities and evaluating health policy. Since 2020, ad hoc only-once online surveys are also used to investigate health services research in primary health care (e.g. corona preparedness survey, Long Covid survey).

The **Belgian SGP network** is a nationwide network comprising about 100 general practices all over Belgium. The coverage of the network is estimated at 1.0% - 1.5% of the Belgian population. On a weekly basis, data is voluntarily reported by the sentinel GPs about 8 different health problems (infectious and non-infectious diseases). Therefore the impact and the

characteristics of different health problems in Belgium can be monitored. Some of them are monitored continuously over time (e.g. acute respiratory infections, influenza-like illness). Other are monitored recurrently (e.g. tick bites and/or suspicion of Lyme disease, advanced care planning) to gather data at regular intervals that can be compared and support the authorities in charge of health policies.

Acute respiratory infections, influenza-like illness and acute diarrhea are covered by the network continuously by weekly reporting of aggregated data (incl. zero reporting). For each episode, age group is recorded. There is also a close collaboration with the National Reference Center for Influenza (NRC Influenza) in Belgium for the virological surveillance in a subset of patients. For the other health topics, case-by-case data are recorded by the sentinel GPs through specific questionnaires in order to better describe the characteristics and risk factors. These questionnaires can be consulted or filled in directly on the website of the SGP network. The registration is done through online forms (LimeSurvey) available on the website of the SGP network. Data is automatically stored on a secured server managed by Sciensano. For more info, see:

- <https://www.sciensano.be/nl/netwerk-van-huisartsenpeilpraktijken> (Dutch)
- <https://www.sciensano.be/fr/reseau-des-medecins-vigies> (French)
- <https://www.sciensano.be/en/network-general-practitioners> (English)

An overview of the health topics monitored during 2025 by the SGP network are presented here:

- Acute respiratory infections
- Influenza-like illness
- Acute diarrhea
- Sexual Transmitted Infections (STIs)
- Tick bites and/or suspicion of Lyme disease
- Varicella
- Zona or post herpetic neuralgia
- Advance Care Planning (ACP)

Ad-hoc only-once questionnaire about:

- Profile of registering practices and GPs in the network: general survey (Summer 2025)
- Long COVID follow-up survey (Summer 2025)

An overview of all publications by the SGP network published during 2025 are available in Annexes.

2. General characteristics of the Sentinel General Practitioners (SGP) network in 2025

2.1. GENERAL CHARACTERISTICS

In 2025, 67 practices corresponding to 95 SGPs of our network belong to the regular (i.c. participating at least 26 weeks per year) participating sentinel GPs (SGPs) in our SGP network.

Table 1. The distribution of the regular SGPs 2025 by region, SGP network.

REGION	N	%
Brussels	11	16.42
Flanders	33	49.25
Wallonia	23	34.33

Table 2. The distribution of the regular SGPs 2025 by practice type, SGP network.

PRACTICE TYPE	N	%
Duo practice	6	8.96
Group practice >2 GPs	13	19.40
Multidisciplinary group practice	26	38.81
Solo practice	22	32.84

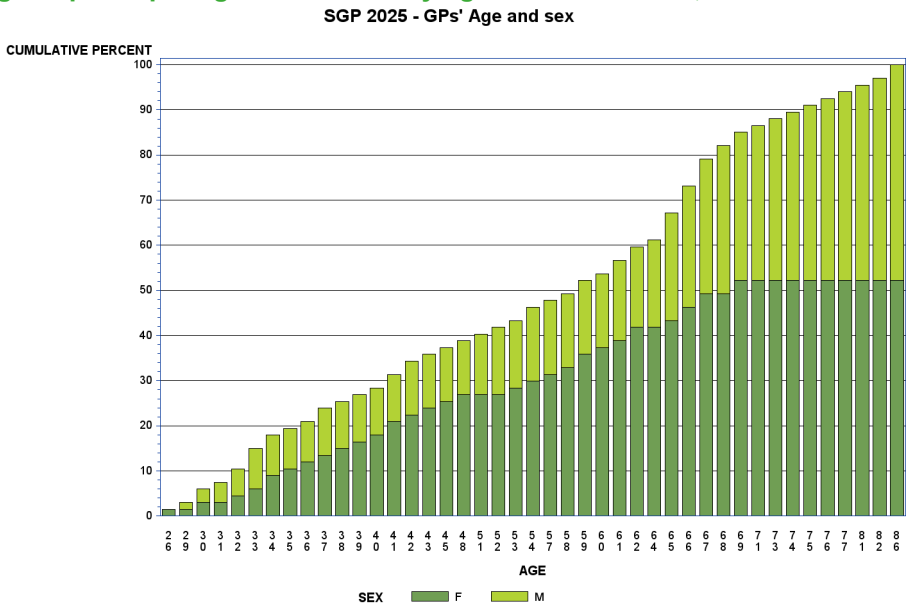
Table 3. The distribution of the regular SGPs 2025 by GP sex, SGP network.

SEX	N	%
Female	35	52.24
Male	32	47.76

The median GP age of the regular SGPs 2025 is 59 years (IQR: 38-67).

Figure 1 presents the distribution of the regular participating sentinel GPs by age and sex in 2025.

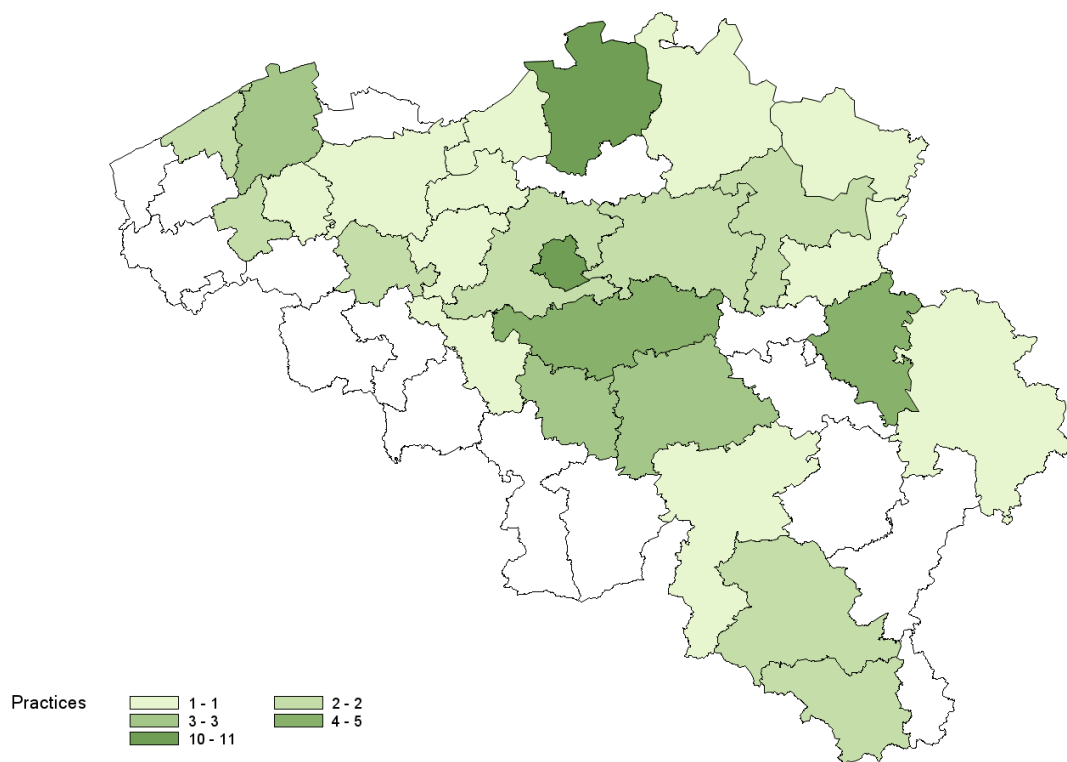
Figure 1. Regular participating sentinel GPs by age and sex in 2025, SGP network.



The following figures present the distribution of the regular participating practices (Figure 2) and regular participating sentinel GPs (Figure 3) in 2025.

Figure 2. Regular participating practices in 2025, SGP network.

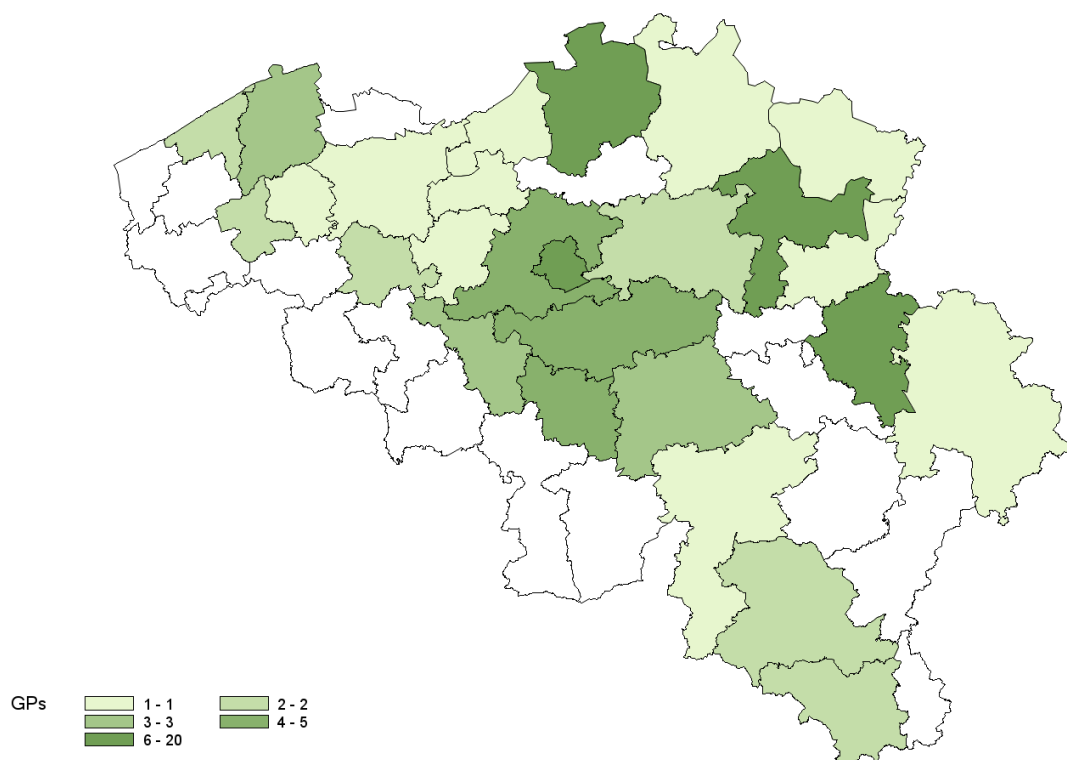
SGP network - regular participating practices in 2025



Generated by Sciensano on 04 March 2026

Figure 3. Regular participating sentinel GPs in 2025, SGP network.

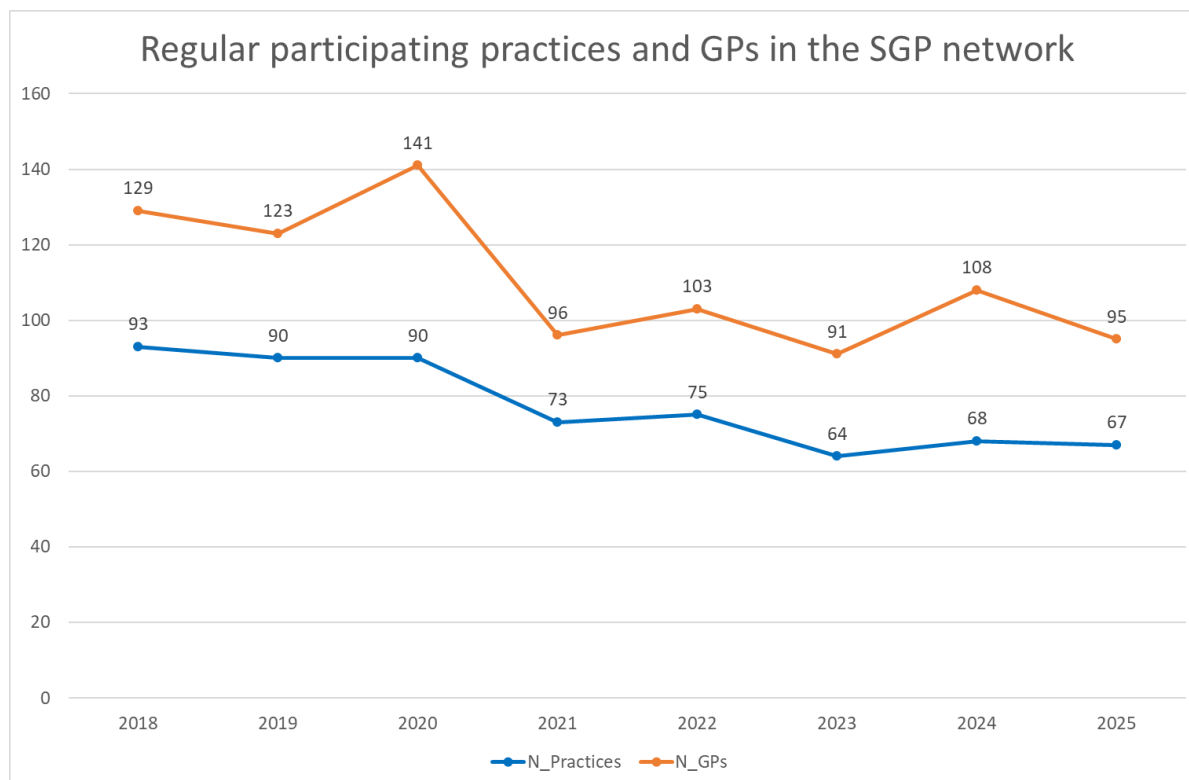
SGP network - regular participating GPs in 2025



Generated by Sciensano on 04 March 2026

Figure 4 gives an overview of the regular participating practices and regular participating sentinel GPs in the SGP network over the years 2018-2025.

Figure 4. Regular participating practices and sentinel GPs in the SGP network, 2018-2025.



2.2. POPULATION COVERAGE

For each year, the population coverage is calculated by district (“arrondissement”), province and region and the total of Belgium. The aim is to reach at least 1% coverage of the Belgian population in every district. An overview of the coverage by the SGP network during 2019-2025 is presented in Table 4.

Formula used to estimate the population covered by the SGP network: the number of inhabitants per active GP (defined as having at least 500 contact patients/year based on health insurance data) was calculated in each of the 43 districts in Belgium and further multiplied by the number of sentinel practices (minimum population coverage) or the number of sentinel GPs (maximum population coverage).

Table 4. Overview of coverage by SGP network, 2019 – 2025.

COVERAGE BY YEAR	Belgium	Brussels	Flanders	Wallonia	N_PRACTICES	N_GPs
COVERAGE1_2025	0,67	1,47	0,54	0,66	67	95
COVERAGE2_2025	0,96	2,13	0,84	0,77	67	95
COVERAGE1_2024	0,70	1,49	0,59	0,62	68	108
COVERAGE2_2024	1,10	2,31	0,95	0,97	68	108
COVERAGE1_2023	0,65	1,42	0,59	0,51	64	91
COVERAGE2_2023	0,93	1,94	0,92	0,60	64	91
COVERAGE1_2022	0,74	1,54	0,73	0,59	75	103
COVERAGE2_2022	1,02	2,45	1,02	0,68	75	103
COVERAGE1_2021	0,73	1,09	0,72	0,67	73	96
COVERAGE2_2021	0,96	1,36	1,05	0,70	73	96
COVERAGE1_2020	0,95	1,21	0,94	0,93	90	141
COVERAGE2_2020	1,50	1,75	1,65	1,13	90	141
COVERAGE1_2019	0,95	1,35	0,90	0,96	90	123
COVERAGE2_2019	1,31	1,89	1,30	1,17	90	123

Coverage 1 and sentinel population 1 is calculated on the number of participating *practices*.

Coverage 2 and sentinel population 2 is calculated on the number of participating *GPs*.

N_Practices and N_GPs refer to the regular participating practices/GPs in our SGP network.

Note: info of 2025 is based on number of GPs in 2024, received from RIZIV/INAMI because no recent info of 2025 available (February 2026)

Note: info of 2024 is based on number of GPs in 2023, received from RIZIV/INAMI because no recent info of 2024 available (February 2025)

Note: info of 2023 is based on number of GPs in 2022, received from RIZIV/INAMI because no recent info of 2023 available (February 2024)

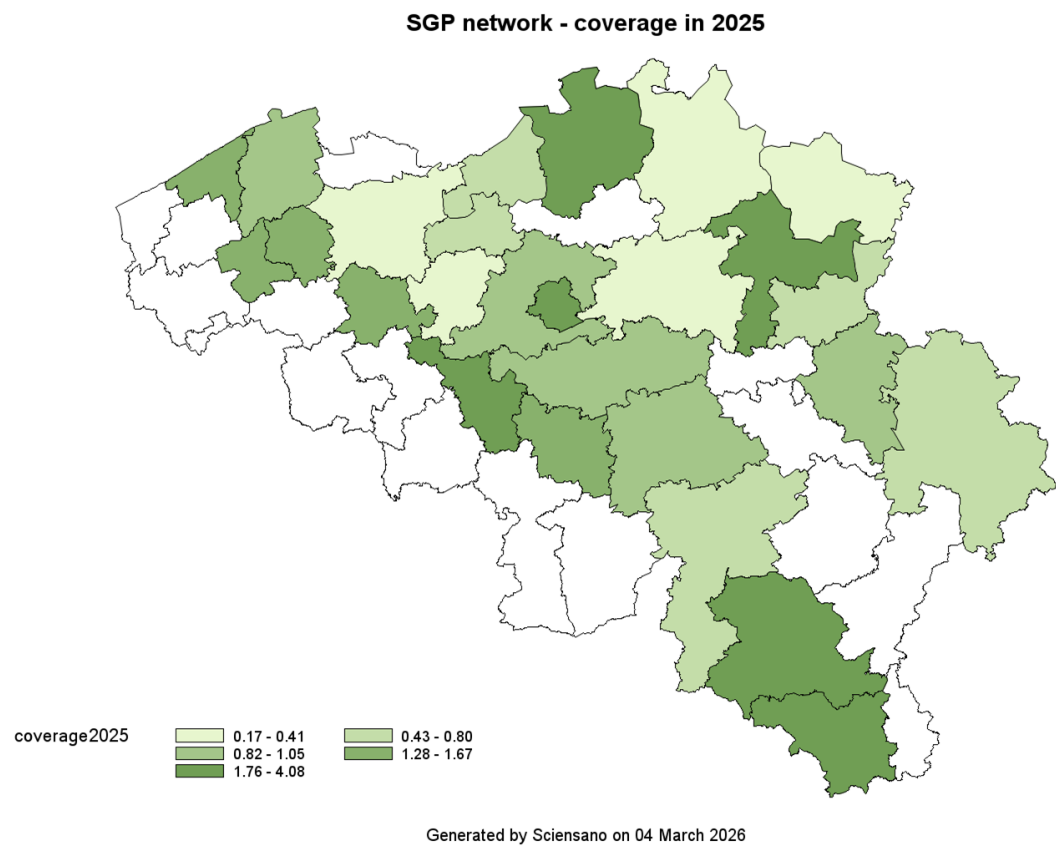
Note: info of 2022 is based on number of GP's in 2021, received from RIZIV/INAMI because no recent info of 2022 available (February 2023)

Note: info of 2021 is based on number of GP's in 2020, received from RIZIV/INAMI because no recent info of 2021 available (February 2022)

Note: info of 2020 is based on number of GP's in 2019, received from RIZIV/INAMI because no recent info of 2020 available (February 2021)

The following figure (Figure 5) presents the distribution of the coverage of the SGP network in 2025.

Figure 5. Coverage of the SGP network in 2025.



3. Recruitment efforts during 2025

During 2025, various recruitment efforts were made to increase the number of participating GPs in the SGP network. A brief overview is giving underneath.

- Visibility of the SGP network:
 - Presentation of the SGP network at national and international conferences/study days
 - Presentation of the SGP network to national GP networks (Domus Medica: promoting the network at events/in newsletter, SSMG: promoting the network at the 'Grandes Journées de la SSMG'/in newsletter)
 - Promoting the network through partners via their newsletters, websites and presentations
 - Collaboration with Collège de Médecine Générale: organisation of a panel discussion and a promotional stand at the 'congrès du CMG'
 - Collaboration with AMUB: Participation in Med G Day, publications, participation in events
 - Publication of an interview in the 'Journal du Médecin'
 - Scientific webinars (with Accreditation System for participating GPs)
 - Creation and distribution of promotional materials (flyers, posters,...)
 - Sending targeted invitation letters to GPs
 - Availability of a network video, see
 - NL version: <https://www.youtube.com/watch?v=5D9UjAfle0I>
 - FR version: <https://www.youtube.com/watch?v=Ki075o7VGMw>
- Changing the methodology of the data collection:
 - Further steps towards reducing the workload for participating GPs: automatic data extraction from the EMD for the weekly aggregated data (implementation from October 2024 onwards) through the implementation of the GP Infection Barometer (from October 2024 onwards). More information on GP Infection Barometer, see <https://www.sciensano.be/en/projects/gp-infection-barometer>
 - Introduction of new methodologies to collect data (for example exploration of a pop-up in the EMD for case-by-case registration, ad hoc only-once online surveys on specific topics, focus groups among GPs, etc.)
- Benefits for the participating GPs:
 - An individual annual report of the sentinel general practices (the results are compared with those of other practices of the network (anonymously))
 - Feedback of individual test results on the samples of patients with an acute respiratory infection
 - A newsletter to inform about the activities, reports and articles of the network
 - Gift vouchers for GPs participating in virological surveillance and very regular GPs
 - Access to scientific webinars (with Accreditation System for participating GPs)
 - Financial compensation of 300€/year for regular participation in weekly and case-by-case registration **or** in the virological surveillance
 - Financial compensation of 400€/year for regular participation in weekly and case-by-case registration **and** in the virological surveillance
 - GPs cited in scientific publications: collaborative group authorship, when applicable
 - Possibility to represent the network in the steering committee: participation in decisions

4. Information collected in the profile survey 2025

During the summer of 2025, a general survey was launched among the SGP network to collect additional information on the profile of the registering practices and GPs in the network.

Two separate surveys were launched, one focussing on the practice info and one focussing on info related to the participating GPs. In total, 70 registering practices (of which 87% belonged to the regular participating practices) responded to the practice info survey and 60 registering GPs answered to the GP info survey.

4.1. PRACTICE INFORMATION COLLECTED IN THE PROFILE SURVEY 2025

In 2025, about two out of three (63%) of the participating practices of the SGP network had administrative support (part-time or full-time) in their practice (Table 5). In half of the practices (52%), GPs in the practice were paid through reimbursement per care activity (Table 6).

Table 5. Response on question ‘Do you have administrative support in your practice?’ (N=70)

Administrative support	N	%	Cumulative N	Cumulative %
Yes, part-time	16	22.86	16	22.86
Yes, full-time	28	40.00	44	62.86
No	26	37.14	70	100.00

Table 6. Response on question ‘How are the doctors in your practice paid?’ (N=70)

Payment method of practice	N	%	Cumulative N	Cumulative %
Reimbursement per care activity	35	50.00	35	50.00
Fees per session	4	5.71	39	55.71
Fixed salary	4	5.71	43	61.43
Performance pay	13	18.57	56	80.00
Fee per capita	5	7.14	61	87.14
Other	9	12.86	70	100.00

Careconnect was the software package mostly used by the participating practices of the SGP network in 2025 (Table 7).

Table 7. Response on question ‘Which software package does your practice work with?’ (N=70)

Used software package	N	%	Cumulative N	Cumulative %
Careconnect (incl. Epicure, Medidoc, Medigest)	35	50.00	35	50.00
Daktari	6	8.57	41	58.57
Health one	10	14.29	51	72.68
Medinect	1	1.43	52	74.29
MediSpring	14	20.00	66	94.29
Omnipro	1	1.43	67	95.71
No answer	3	4.29	70	100.00

In 2025, all practices of the SGP network offered medical services with appointment, and almost all practices offered home visits (94%), while 71% offered telephone consultations (Table 8).

Table 8. Response on question ‘Does your practice offer the following to your patients? (multichoice)’ (N=70)

My practice offers the following to the patients	N	%
Medical services with appointment	70	100.00
Medical services without appointment	13	18.57
Home visits	66	94.29
Consultations by telephone	50	71.43
Video consultations	9	12.86
Other remote consultations (e.g. via e-mail, SMS or patient platforms)	21	30.00

Related to the presence and availability of mental health services in the primary care zone¹ of the practice, 66% of practices mentioned that mental health care services were available but that there were (long) waiting lists (Table 9).

Table 9. Response on question ‘Presence and availability of mental health services in the primary care zone of the practice? (multichoice)’ (N=70)

Presence and availability of mental health services	N	%
Mental health care is present in the primary care area of the practice	22	31.43
Mental health care is present in the primary care zone of the practice, but there are (long) waiting lists	46	65.71
Mental health services are present in the primary care zone of the practice, but they are difficult to reach for patients (distance, transport)	7	10.00
Mental health care is not present in the primary care zone of the practice	9	12.86

In 2025, a question was included about the routinely development of care plans in the practice. About one in three (37%) of the participating practices of the SGP network develops care plans for patients with specific chronic conditions, while almost half (46%) of the practices stated to not routinely develop care plans.

Table 10 • Response on question ‘Are care plans routinely developed in your practice for any of the following patient groups? (multichoice)’ (N=70)

Are care plans routinely developed in your practice for:	N	%
Patients with chronic conditions	18	25.71
Patients with specific chronic conditions	26	37.14
Patients with specific needs (e.g. patients with dietary requirements or complex care needs)	7	10.00
Other patients	2	2.86
Care plans are not routinely developed	32	45.71
No answer	4	5.71

¹ In Dutch: eerstelijnszone / in French: zone de soins de première ligne

4.2. GP INFORMATION COLLECTED IN THE PROFILE SURVEY 2025

For almost all (92%) GPs of the SGP network, the main reason for GPs to participate in the SGP network was scientific interest, followed by 37% also mentioning the individual annual benchmark feedback report.

In 2025, 43% of the participating GPs of the SGP network were working more than 30 years as GP in a practice (Table 11).

Table 11. Response on question 'Number of years working (full-time or part-time) as a general practitioner in a practice' (N=60)

Years working as GP in a practice	N	%	Cumulative N	Cumulative %
Less than 5 years	5	8.33	5	8.33
5 to 10 years	11	18.33	16	26.67
10-19 years	12	20.00	28	46.67
20-29 years	6	10.00	34	56.67
More than 30 years	26	43.33	60	100.00

In 2025, 65% of the participating GPs of the SGP network had no professional affiliation with an elderly care institution or residential care centre, 22% had a professional connection but was not a CRA (coordinating and advisory physician) in the residential care centre and 13% was working as CRA in the residential care centre.

In 2025, of the participating GPs of the SGP network:

- 5% had a professional connection with a mental health care centre
- 5% had a professional connection with a reception centre for persons with disabilities
- 8% had a professional connection with an asylum seekers' centre
- 2% had a professional connection with a prison
- 2% had a professional connection with a family planning centre
- 2% had a professional connection with a prevention and screening centre for HIV, sexually transmitted infections and viral hepatitis
- 3% had a professional connection with a centre for drug users

In 2025, 93% of the participating GPs of the SGP network recorded at least one home visit in the electronic medical patient record. The participating GPs of the SGP network were asked how frequently they used diagnostic codes or standardized coding (via thesaurus, etc.) to record registering topics in the electronic medical patient record. The following percentage of participating GPs mentioned to *always* use diagnostic codes or standardized coding for:

- acute respiratory infections: 52% (N=31)
- influenza-like illness: 48%(N=29)
- acute diarrhea: 43% (N=26)
- sexually transmitted infections: 45% (N=27)
- varicella, zona or postherpetic neuralgia: 62% (N=37)
- tick bites and/or suspicion of Lyme disease: 48% (N=29)
- advance care planning: 12% (N=7)
- hepatitis B and/or C viruses: 42% (N=25)
- mental health problems: 22% (N=13)

The following percentage of participating GPs of the SGP network had recently (within the last 3 years) taken a course in:

- Sexual health or STIs: 45% (N=27)
- Lyme disease: 23% (N=14)
- Infectious diseases: 58% (N=35)
- Substance misuse (e.g. alcoholism): 42% (N=25)
- (Rational use of) psychotropic drugs: 48% (N=29)
- Mental health (e.g. depression, burnout): 53% (N=32)
- Violence: 22% (N=13)
- Smoking: 18% (N=11)
- Geriatrics / falls in older people: 47% (N=28)
- End-of-life care, advance care planning or palliative care: 60% (N=36)
- Cardiovascular conditions: 62% (N=37)
- Oncology: 35% (N=21)
- Rare diseases: 15% (N=9)

In 2025, some specific questions focusing on violence were added. The following percentage of participating GPs mentioned to *often* encounter situations in which the patient is a victim of violence:

- Psychological abuse: 27% (N=16)
- Physical abuse: 3% (N=2)
- Sexual abuse: 2% (N=1)
- Economic abuse: 10% (N=6)

The participating GPs of the SGP network mentioned that this *often* occurred in these contexts:

- Within a relationship: 42% (N=25)
- Within the family: 33% (N=20)
- At work or at school: 23% (N=14)
- In an institution (e.g. care home, centre for people with disabilities, asylum seekers' centre, prison): 5% (N=3)
- Public place: 7% (N=4)

In 2025, 42% of the participating GPs of the SGP network mentioned to have encountered less than 10 violence cases in the past year, while 53% stated to have encountered much more violence cases in the past year (Table 12).

Table 12. Response on question 'How many violence cases have you encountered in the past year (patient complaints or suspected cases)?' (N=60)

	N	%	Cumulative N	Cumulative %
No cases	2	3.33	2	3.33
Yes, fewer than 10 cases	25	41.67	27	45.00
Yes, between 10 to 30 cases	23	38.33	50	83.33
Yes, between 30 to 50 cases	6	10.00	56	93.33
Yes, more than 50 cases	3	5.00	59	98.33
No answer	1	1.67	60	100.00

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ANNEXES

Publications by the SGP network during 2025

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